Prior Authorization Request Packet

Case pa_f61383b0 · Aetna · MRI knee (lower extremity joint) without contrast (73721) · Policy Aetna CPB 0171
Generated 2026-10-11T23:47:56.055Z

1. Letter of medical necessity

October 11, 2026

Aetna, Utilization Management / Prior Authorization

RE: Marcus Lindqvist, DOB 1991-06-23, Member ID AET-513980027
Requested service: MRI knee (lower extremity joint) without contrast, CPT 73721

To Whom It May Concern:

I am writing on behalf of Dr. Priya Raman to request prior authorization for a right knee MRI without contrast for Mr. Lindqvist, a "35-year-old male recreational soccer player" with "persistent right knee pain and intermittent swelling". The note records: "He recalls a twisting injury during a game earlier this summer." It continues: "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs." Examination documented: "Moderate right knee effusion." "Medial joint line tenderness." "McMurray test positive medially with a palpable click." "Lachman negative, posterior drawer negative." and "Range of motion 5 to 120 degrees, limited by pain." The assessment is: "Suspected right medial meniscus tear with persistent effusion and instability."

Policy review: Aetna CPB 0171 (https://www.aetna.com/cpb/medical/data/100_199/0171.html).

Criterion 1, persistent knee pain, swelling, and/or instability: Met. The note states: "Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

Criterion 2, at least 3 weeks of conservative therapy without response: Partly documented, and I want to be precise. The note records "He has been doing physical therapy", "taking ibuprofen" and "has been using a brace", but gives no duration. Per telephone confirmation with the ordering physician, treatment has lasted "about five, six weeks". The same call transcript also states "it wasn't along with us any compression or any elevation". The policy defines conservative therapy as rest, ice, compression and elevation plus NSAIDs and/or physical therapy. The NSAID and physical therapy components are documented and the stated duration exceeds 3 weeks. However, the physician's statement indicates compression and elevation were not part of the regimen, and the record does not document rest or ice. The call transcript is also imperfect, and the physician may wish to clarify it.

Criterion 3, multi-view x-rays ruling out fracture or loose body: Met. The note records: "Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

Alternative indications in the policy: Persistent true locking is not met; the note states "He denies true locking of the knee." Suspected osteomyelitis or septic joint is not documented; the note records "No fever, no redness." and "No warmth or erythema." Tumor evaluation is not documented and is not the basis of this request.

Conclusion: Mr. Lindqvist has persistent pain, swelling and giving way, a positive medial McMurray test with effusion, and negative plain films for fracture or loose body. Criteria 1 and 3 are met. Criterion 2 is met as to duration and the NSAID and physical therapy components, but the compression, elevation, rest and ice elements are not confirmed. The note plans to "Continue physical therapy pending imaging." I respectfully request authorization of CPT 73721 so that the suspected medial meniscus tear can be evaluated. Please contact our office if further documentation is needed.

Sincerely,

Priya Raman, MD
NPI 1891234567
Ordering Physician

2. Criteria checklist with verified evidence

MET aetna-73721-c1: Persistent knee pain, swelling, and/or instability (giving way).
"Since then he has had medial knee pain, swelling after activity, and a sensation of the knee giving way on stairs."

MET aetna-73721-c2: Not responding to at least 3 weeks of conservative therapy (conservative therapy consists of a combination of rest, ice, compression, elevation, plus NSAIDs and/or physical therapy).
"the patient has been getting the medical treatment about five, six weeks" [phone_call]
code check: 5.5 weeks documented vs 3 required

MET aetna-73721-c3: Multi-view x-rays have been obtained and have ruled out a fracture or loose body in the knee.
"Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint spaces preserved, small effusion."

MISSING aetna-73721-r1: Persistent true locking of the knee indicative of a torn meniscus or loose body (more than a momentary locking of the joint with the knee in a flexed position).

MISSING aetna-73721-r2: Suspected bone infection (i.e., osteomyelitis) or septic joint.

MISSING aetna-73721-r3: Detection, staging, and post-treatment evaluation of tumor of the knee.

3. Source visit note

```
CHIEF COMPLAINT: Right knee pain and swelling.

HISTORY OF PRESENT ILLNESS: 35-year-old male recreational soccer player presents with
persistent right knee pain and intermittent swelling. He recalls a twisting injury during a
game earlier this summer. Since then he has had medial knee pain, swelling after activity,
and a sensation of the knee giving way on stairs. He denies true locking of the knee. No
fever, no redness. He has been doing physical therapy and taking ibuprofen, and has been
using a brace, but the symptoms have persisted.

PHYSICAL EXAM: Moderate right knee effusion. Medial joint line tenderness. McMurray test
positive medially with a palpable click. Lachman negative, posterior drawer negative. Range
of motion 5 to 120 degrees, limited by pain. No warmth or erythema.

IMAGING: Right knee x-rays, 3 views, obtained 09/15/2026: no fracture, no loose body, joint
spaces preserved, small effusion.

ASSESSMENT: Suspected right medial meniscus tear with persistent effusion and instability.

PLAN: MRI right knee without contrast (CPT 73721) to evaluate meniscus and ligaments.
Continue physical therapy pending imaging.
```

4. Telephone confirmation with ordering physician

Call cmv4h13pg02950foxtuiq2sah · 2026-10-11T23:45:31.982Z · 73 s

agent: Hi, this is the prior-authorization assistant from the clinic. Am I speaking with Dr. Priya Raman?
physician: Yeah. Yeah. I'm doctor Robin.
agent: Thank you, Doctor.

I'm calling about your patient Marcus Lindqvist. Aetna requires prior authorization for an MRI knee (lower extremity joint) without contrast, and under Aetna CPB 0171 the visit note is missing one detail. This should only take a minute.

First question:
How long has the patient been doing physical therapy and taking ibuprofen, and was that for at least three weeks, along with rest, ice, compression, and elevation?
physician: So if I'm not mistaken, the patient has been getting the medical treatment about five, six weeks from I was twenty two October first. And it wasn't along with us any compression or any elevation.
agent: Got it, so about five to six weeks of treatment starting around September 22nd or October 1st, and without rest, ice, compression, or elevation. Thank you, that's everything I needed. We'll finish the packet and submit it today. Have a good day.